

Competitive Landscape: Daraxonrasib (Rasonque) — Revolution Medicines

Segment: Pan-RAS / KRAS-targeted oncology — pancreatic ductal adenocarcinoma (PDAC) and adjacent indications **Analysis date:** September 2026 **Prepared by:** VigilLine · **Prepared from:** public sources only · 25 cited sources · 10 sections

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1. Executive Summary

Daraxonrasib (brand Rasonque, Revolution Medicines, Nasdaq: RVMD) is the first broadly mutation-agnostic RAS(ON) inhibitor to reach the market. It is an oral, once-daily, non-covalent tri-complex molecular glue that binds cyclophilin A intracellularly and sterically blocks the GTP-bound (active) state of mutant and wild-type RAS across KRAS, NRAS, and HRAS — covering G12, G13, and Q61 codon mutations without a companion diagnostic.[1][2][3]

In the pivotal Phase 3 RASolute 302 trial (N=500, 91.8% RAS G12-mutant), daraxonrasib delivered:

- Median OS **13.2 months vs 6.7 months** for investigator's-choice chemotherapy (HR 0.40, $p < 0.0001$) — a 60% reduction in risk of death.[4][5]
- Median PFS **7.2 months vs 3.6 months** (HR 0.49, $p < 0.0001$).[4]
- ORR **31.6% vs 11.2%** (central review).[4]
- Grade ≥ 3 treatment-related AEs: 43.6% (daraxonrasib) vs 57.5% (chemo); discontinuations due to TRAEs: 1.2% vs 11.2%.[4]
- Published in NEJM (NCT06625320).[5]

The drug is priced at **\$39,800 per 30-day supply**. [6] Revolution Medicines carries a **~\$45B market cap**, roughly \$11.5M in recent revenue, and a peak-sales estimate around **\$11.5B** for daraxonrasib alone.[6][7]

The competitive field is a fast-moving multi-modality race across **744 active drug programs, 2,355 patent filings since Jan 2023, and 123 ongoing Phase 2/3 trials** targeting KRAS/RAS.[8] Daraxonrasib's differentiation is its **pan-RAS, mutation-agnostic coverage** — but it faces direct competition from allele-specific KRAS G12D programs (Astellas, Hengrui, Theras), a competing pan-RAS platform, and longer-term threats from PROTAC degraders, vaccines, and TCR-T cell therapies.

2. The Target: Why RAS Matters

- KRAS mutations occur in ~25% of all human cancers: **~90% of PDAC**, ~40% of colorectal, ~30% of NSCLC, and ~13% of NSCLC harbor the G12C subtype.[8][9]
 - PDAC specifically: ~510,000 new diagnoses and ~470,000 deaths globally per year; 5-year survival ~12%; 80-85% present with locally advanced/metastatic disease.[9]
 - Historically "undruggable" — the G12C covalent approach (sotorasib, adagrasib) opened the door but covers only ~13% of NSCLC and 3-4% of CRC; the dominant PDAC mutations (G12D ~45%, G12V ~30%) remain without approved targeted therapy.[8][9]
 - Daraxonrasib's tri-complex mechanism bypasses the need for a cysteine handle, allowing it to engage **wild-type and multiple mutant RAS variants** including G12D, G12V, and G12R.[3][10]
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3. Daraxonrasib — Product & Company Profile

Revolution Medicines (RVMD)

- **HQ:** Redwood City, CA; founded 2014; ~1,261 employees.[11]
- **Stage:** Public (Nasdaq: RVMD); ~\$45B market cap (intraday); YTD return +163%, 1-year +439%.[6]
- **Revenue:** ~\$11.5M (latest); pre-commercial, R&D-driven; Q2 FY26 net loss ~\$644M.[6]
- **Pipeline:** RAS(ON) inhibitor platform with multiple allele-selective and multi-selective programs:[12]
- **Daraxonrasib (RMC-6236)** — pan-RAS(ON) multi-selective; approved 2L PDAC; Phase 3 in 1L PDAC, NSCLC 2L/3L, adjuvant resectable.[12]
- **Zoldonrasib (RMC-9805)** — KRAS G12D-selective covalent; Phase 1/2; Phase 3 combo with daraxonrasib in 1L G12D-mutant PDAC (RASolute 309, NCT07805954).[13][14]
- **Elironrasib (RMC-6291)** — KRAS G12C-selective; early clinical.[12]
- **RMC-5127** — KRAS G12V-selective; clinical.[12]
- **RMC-0708 (Q61H), RMC-8839 (G13C)** — early development.[12]
- Legacy: RMC-4630 (SHP2 inhibitor), RMC-5552 (mTORC1 inhibitor).[11]

Daraxonrasib Details

- **Mechanism:** Oral, non-covalent, tri-complex molecular glue; binds cyclophilin A → binary complex → engages GTP-bound RAS(ON) → steric blockade of effector binding; also stimulates GTP hydrolysis of codon 12 mutants.[10][15]
- **Indications (approved):** Adult patients with metastatic PDAC who have received ≥1 prior systemic therapy or who are not candidates for multiagent chemotherapy.[1][2]
- **Indications (investigational):** 1L metastatic PDAC (RASolute 309 combo with zoldonrasib vs gem/nab-paclitaxel, NCT07805954); NSCLC 2L/3L; adjuvant resectable PDAC; solid tumors + combination doublets.[12]

- **Safety:** Dermatologic toxicities in 86% (10% Grade 3) — papulopustular rash, pruritus, dry skin, paronychia, fissures; stomatitis 57% (9% Grade 3); diarrhea 63% (6% Grade 3). Prophylactic topical corticosteroids, emollients, sunscreen, oral doxycycline recommended at initiation.[1]
- **Regulatory path:** FDA accepted NDA July 22, 2026; granted approval Aug 26, 2026; Commissioner's National Priority Voucher used.[16][1]
- **Partnership:** Revolution Medicines + BeOne Medicines — clinical development and regional commercialization collaboration announced Aug 10, 2026.[17]

4. Competitive Field — By Mechanism Class

4.1 KRAS G12C Selective Inhibitors (Saturated — 6 Global Approvals)

Company	Drug	Approval	Notes
Amgen	Sotorasib (Lumakras)	FDA 2021	First-in-class; CodeBreak 200 Ph3 missed PFS vs docetaxel; still on accelerated approval; CRC combo with panitumumab Ph3 ongoing.[8]
Mirati / BMS	Adagrasib (Krazati)	FDA 2022	BMS acquired Mirati for \$5.8B (2024); CRC combo with cetuximab approved June 2024; 1L NSCLC Ph3 with pembro+chemo.[8]
Innovent (China)	Fulzerasib (GFH925)	NMPA Aug 2024	Second China G12C approval.[8]
InventisBio (China)	Garsorasib (D-1553)	NMPA Nov 2024	KRAS G12C; InxMed combo with FAK inhibitor IN10018 Ph3 in NSCLC. [8][18]
Jacobio (China)	Glecirasib (JAB-21822)	NMPA May 2025	Ex-Asia rights to AbbVie (~\$1B+ deal 2023).[8]
(China)	Sosimerasib	NMPA Feb 2026	Latest China G12C approval.[8]

- **Strategic position:** Saturated class. Differentiation now through combinations, earlier-line positioning, and resistance management, not first-in-class.[8]
- **Relevance to daraxonrasib:** G12C inhibitors do NOT cover the dominant PDAC mutations (G12D, G12V). They are direct competitors only in the NSCLC and CRC overlap where G12C is prevalent — and there daraxonrasib's pan-RAS coverage could be an advantage if it proves active in G12C-mutant NSCLC.

4.2 KRAS G12D Selective Programs (Most Actively Contested Space)

G12D is the dominant PDAC mutation (~45% of KRAS-mutant PDAC). No approved G12D therapy exists.[8][9]

Astellas / Cullgen — Setidegrasib (ASP3082)

- **Mechanism:** First-in-class KRAS G12D-targeted **PROTAC degrader** — binds mutant G12D and an E3 ligase, triggering proteasomal degradation.[19]
- **Status:** Phase 3 in 1L KRAS G12D-mutant metastatic PDAC (NCT07409272) — first patient dosed April 15, 2026; randomized, double-blind, placebo-controlled; >600 patients planned; primary endpoint OS; combination with mFOLFIRINOX or NALIRIFOX.[19][20]
- **Phase 1 readout (NEJM, March 2026):** In 21 PDAC patients at 600mg (2L/3L), ORR 24% (95% CI 8-47%), mPFS 3.0 months (95% CI 1.4-6.9), mOS 10.3 months (95% CI 4.2-13.0).[21]
- **Also:** Phase 2 in KRAS G12D-mutant NSCLC.[19]
- **Positioning:** Direct competitor to daraxonrasib in PDAC but **1L combination** strategy vs daraxonrasib's current 2L monotherapy — different treatment line. PROTAC mechanism may overcome resistance that limits small-molecule G12D inhibitors; manufacturing complexity is a commercial constraint.[8][19]
- **Also in Revolution's crosshairs:** Revolution's own zoldonrasib (RMC-9805) is a G12D-selective molecular glue entering Phase 3; the RASolute 309 trial combines zoldonrasib + daraxonrasib in 1L G12D PDAC — a direct head-to-head domain against Astellas' approach.[13][14]

Hengrui (China) — HRS-4642 - KRAS G12D small molecule; multiple Phase 1/2 programs; also covers G12C.[8] - Tier 2 — Active; Chinese player in the G12D space.[8]

Theras (Boundless Bio) — TSN-1611 - KRAS G12D small molecule; Phase 1; 4 patents since 2023.[8] - Tier 2 — Active.

Genfleet Therapeutics — GFH276 - KRAS G12D molecular glue; Phase 1 in PDAC and CRC.[9]

Erasca — ERAS-0015 - Pan-KRAS inhibitor; Phase 1 in KRAS-mutant solid tumors.[9]

Chinese G12D ecosystem: Sichuan Huiyu Pharma (8 patents 2023+), Betta Pharmaceutical (7), Leading Pharmaceutical Shaoxing (6) — chemistry platform players.[8]

4.3 Pan-RAS / Multi-Selective Inhibitors (Daraxonrasib's Home Turf)

- **Revolution Medicines — Daraxonrasib (RMC-6236):** The only approved and Phase 3-advanced pan-RAS(ON) inhibitor. Fastest-growing differentiated route; single-molecule coverage of multiple RAS mutations could displace mutation-selective inhibitor sequencing if Phase 3 data holds.[8]
- **Preclinical/early-stage competitors:** Multiple discovery-stage pan-RAS programs from academic and Chinese groups; no other Phase 3 pan-RAS entrant.[8]
- **RMC-6236 preclinical tool compound RMC-7977:** Preclinical studies show that combining with ICIs or CDK4/6 inhibitors confers deeper, more durable regressions in PDAC models; triple therapy with afatinib + STAT3 PROTAC also showed robust regression.[15]

4.4 KRAS G12C Next-Generation (Competitive in NSCLC/CRC Overlap)

Eli Lilly — Olomorasib (LY3537982) - Second-gen KRAS G12C inhibitor; Phase 2; 1L NSCLC Ph3 with pembrolizumab (NCT06119581).[8][18]

Roche / Genentech — Divarasib (GDC-6036) - KRAS G12C; Phase 3 vs docetaxel in 2L NSCLC (NCT06497556); Ph3 with cetuximab in CRC (NCT06793215); MORPHEUS combination studies.[8][18]

Merck — Caldasiasib (MK-1084) - KRAS G12C; Phase 3 with pembrolizumab in 1L NSCLC (NCT06345729); adjuvant Ph3 (NCT07431827).[18]

- **Relevance:** These compete with daraxonrasib only in the NSCLC indication where G12C is common. Daraxonrasib has a Phase 3 registrational trial in 2L/3L NSCLC — it will compete directly with these agents in that setting, where pan-RAS coverage may offer an edge in non-G12C patients but faces tougher competition in the G12C subset.

4.5 PROTAC Degraders (Beyond G12D)

Company	Asset	Target	Status
Astellas/Cullgen	Setidegrasib (ASP3082)	KRAS G12D	Phase 3 (PDAC, NSCLC)
Arvinas + partners	Multiple	KRAS G12C, broad	Discovery/preclinical
Astellas	Additional KRAS PROTACs	Various	Follow-on programs[8]

- 24 PROTAC degrader programs across the KRAS field; Astellas leads with the only Phase 3 entrant.[8]

4.6 Vaccines & Cell Therapy (Longer-Term Disruption Threat)

Modality	Asset	Sponsor	Status	Target
mRNA vaccine (personalized)	mRNA-4157 (Intismeran)	Moderna/Merck	Phase 3 adjuvant melanoma; PDAC trials	Shared + personalized neoantigens
mRNA vaccine (shared KRAS)	mRNA-4359 / mRNA-5671	Moderna	Phase 1/2	KRAS G12C/D/V/G13D peptides
Amphiphile peptide vaccine	ELI-002	Elicio Therapeutics	Phase 2 adjuvant PDAC, CRC	Shared KRAS neoantigens, lymph-node targeted[9]
Personalized neoantigen mRNA vaccine	BNT122 (autogene cevumeran)	BioNTech/Genentech	Phase 2 adjuvant PDAC (IMCODE-004 randomized)[9]	Patient-specific neoantigens
TCR-T cell therapy	27 programs	Multiple (academic + Chinese)	Discovery/early clinical	KRAS hotspot neoantigens (G12V, G12D, G12C peptides on HLA-A11/A02)[8]

- **Threat horizon:** Vaccines and TCR-T could transform KRAS management by 2030; currently adjuvant/MRD settings; not direct competitors to daraxonrasib's current 2L metastatic indication but could compress the long-term opportunity if they move into earlier lines.[8][9]

4.7 Other PDAC Mechanism Clusters (Contextual Competition)

Class	Examples	Status
Chemotherapy combinations (1L)	NALIRIFOX, FOLFIRINOX, gemcitabine + nab-paclitaxel	Standard of care; NAPOLI 3 established NALIRIFOX superiority (mOS 11.1 vs 9.2 months)[9]
PARP inhibitor (BRCA-mutant maintenance)	Olaparib (Lynparza)	FDA 2019 (POLO trial)[9]
Claudin 18.2 ADC/mAb	Zolbetuximab (Vyloy)	FDA-approved gastric; PDAC trials underway[9]
TGF- β mAb	NIS793 + canakinumab	Phase 3 missed[9]
PD-1/VEGF bispecific	Ivonescimab + chemo	Phase 2/3 1L mPDAC + NALIRIFOX[9]
Immunotherapy (MSI-H only)	Pembrolizumab	FDA-approved 2L MSI-H/dMMR PDAC (not EMA-approved)[22]

5. Competitive Positioning Matrix

By indication (current and near-term)

Indication	Daraxonrasib status	Key competitors	Competitive intensity
2L+ metastatic PDAC	APPROVED (Aug 2026) — first targeted therapy	Chemo only (FOLFIRI, gem+nab-paclitaxel, FOLFOX); mOS ~6-6.5 months historically[9][23]	Low — daraxonrasib is the only targeted option; clear standard-of-care winner
1L metastatic PDAC (G12D-mutant)	Phase 3 (RASolute 309: daraxonrasib + zoldonrasib vs gem/nab-paclitaxel, NCT07805954)[14]	Setidegrasib + mFOLFIRINOX/NALIRIFOX (Astellas, NCT07409272)[19]; zoldonrasib monotherapy; GFH276; HRS-4642	High — direct G12D battle; 1L is the largest commercial segment
1L metastatic PDAC (all-comers)	Phase 3 monotherapy registrational trial[12]	NALIRIFOX (standard); future G12D/pan-RAS entrants	Medium — chemotherapy is entrenched; biomarker strategy matters
NSCLC 2L/3L (RAS-mutant)	Phase 3 registrational[12]	Olomorasib, divarasib, calderasib (G12C-focused); adagrasib, sotorasib	High — large market, G12C incumbents, pan-RAS could differentiate in non-G12C subgroup
Adjuvant resectable PDAC	Early clinical[12]	ELI-002, BNT122, mRNA-5671 (vaccines); future G12D/pan-RAS	Low now — early stage, vaccines are the lead modality here
CRC (RAS-mutant)	Early clinical[12]	G12C inhibitors + EGFR mAb (sotorasib+pani, adagrasib+cexu); amivantamab (Janssen)[18]	Medium — G12C class has traction; pan-RAS could expand addressable population

By strategic position (tier ranking, from Eureka/Patsnap analysis)[8]

Tier	Player	Lead asset	Strategic position
Tier 1 — Leader	Revolution Medicines	Daraxonrasib (pan-RAS)	Highest-novelty route; only approved pan-RAS; multi-mutation coverage; fastest-growing segment
Tier 1 — Leader	Amgen	Sotorasib (G12C)	First-in-class G12C; established commercial scale; but CodeBreak 200 disappointment
Tier 1 — Leader	Mirati/BMS	Adagrasib (G12C)	Longer half-life, CNS penetration; \$5.8B BMS acquisition validates KRAS value
Tier 1 — Leader	Astellas/Cullgen	Setidegrasib (G12D PROTAC)	Sole Phase 3 G12D entrant; differentiated degradation mechanism; direct PDAC competitor
Tier 1 — Leader (China)	Innovent, InventisBio, Jacobio	Fulzerasib, Garsorasib, Glecirasib	Four NMPA-approved G12C; regional ecosystem maturity; AbbVie ex-Asia deal validates commercial credibility
Tier 2 — Active	Eli Lilly	Olomorasib (G12C next-gen)	Second-gen G12C; 1L NSCLC Ph3; competing to displace first-gen
Tier 2 — Active	Roche/Genentech	Divarasib (G12C)	Phase 3 NSCLC + CRC; MORPHEUS combinations
Tier 2 — Active	Hengrui	HRS-4642 (G12D)	Chinese G12D small molecule; multiple Phase 1/2
Tier 2 — Active	Theras/Boundless Bio	TSN-1611 (G12D)	Phase 1; 4 patents
Tier 2 — Active	Moderna/Merck	mRNA-4157, mRNA-4359	mRNA vaccine platform; melanoma adjuvant Ph3; KRAS vaccines emerging
Tier 2 — Active	Genfleet	GFH276 (G12D)	Phase 1 PDAC + CRC
Tier 2 — Active	Erasca	ERAS-0015 (pan-KRAS)	Phase 1
Tier 3 — Emerging	Elicio	ELI-002 (KRAS peptide vaccine)	Phase 2 adjuvant PDAC; amphiphile platform
Tier 3 — Emerging	BioNTech/Genentech	BNT122 (personalized mRNA vaccine)	Phase 2 adjuvant PDAC; randomized IMCODE-004
Tier 3 — Emerging	TCR-T programs (27)	Various	Neoantigen recognition; long-term disruption potential

6. Patent & IP Landscape

- **2,355 patent filings** since January 2023 across the KRAS field; **744 active drug programs**; **123 ongoing Phase 2/3 trials**.^[8]
- **Top assignees by patent volume (2023+)**: Sichuan Huiyu Pharma (8), Betta Pharmaceutical (7), Mirati/BMS (7), Leading Pharmaceutical Shaoxing (6), Khr Biotec (6), Theras (4), Leidos/Lawrence Livermore/NCIFrederick (4), Shanghai Allist (4), Risen Suzhou (4).^[8]
- **Structural signal**: Chinese chemistry platforms dominate patent volume; U.S. biotechs (Mirati/BMS, Theras) and the NCIFrederick collaboration (Leidos + Lawrence Livermore) are the non-Chinese

standouts. KRAS G12D and pan-RAS routes remain Western-dominated (Astellas, Revolution Medicines) — reflecting higher chemistry complexity.[8]

- **Revolution Medicines IP:** Composition claims for daraxonrasib (RMC-6236) plus follow-on KRAS G12D (zoldonrasib/RMC-9805) and allele-selective programs; tri-complex molecular glue mechanism is the key differentiator.[8][12]
- **BD landscape:** BMS-Mirati \$5.8B acquisition is the largest KRAS deal; AbbVie-Jacobio ~\$1B Glecirasib ex-Asia licensing validates Chinese asset commercial credibility.[8]

7. Unmet Needs & White Spaces

Unmet need	Current approaches	Gap	White space
KRAS G12D-mutated PDAC	Setidegrasib Ph3, zoldonrasib Ph1/2, HRS-4642, GFH276, TSN-1611	No approved G12D therapy	PROTAC + small molecule sequencing; 1L combination strategy
Pan-RAS coverage (multi-mutation)	Daraxonrasib (only Phase 3/approved)	Limited approved options	Pan-RAS + IO combinations; adjuvant expansion; NSCLC/CRC extension
KRAS G12C resistance management	Multiple combination trials	Limited approved post-progression options	Sequential G12C → pan-RAS or PROTAC; combination regimens
Adjuvant minimal residual disease	mRNA-4157, ELI-002, BNT122 vaccines	No approved adjuvant KRAS therapy	Vaccine + PD-1 + selective inhibitor combinations
Biomarker-guided patient selection	ctDNA, KRAS allele frequency, BRCA, CLDN18.2, MSI, neoantigen burden — all inconsistent	No consistent clinical implementation	Companion diagnostic + comprehensive genomic profiling as strategic moat[9]

8. Risks to Daraxonrasib's Competitive Position

1. **Astellas setidegrasib Phase 3 in 1L G12D PDAC:** If positive, offers a competing mechanism (PROTAC degradation) with a different treatment-line strategy. The G12D segment is ~40% of PDAC — the largest single-allele slice. Revolution's counter is zoldonrasib + daraxonrasib combo (RASolute 309).[13][14][19]
2. **Pan-RAS toxicity concerns:** Wild-type RAS inhibition raises theoretical on-target normal-tissue effects. Daraxonrasib's safety data so far are manageable (dermatologic + GI), but long-term chronic-use tolerability in a broader population remains unproven. There is a therapeutic window — normal cells are less RAS-dependent — but this is a key watch item.[1][10][15]
3. **Resistance:** Preclinical studies with RMC-7977 (daraxonrasib tool compound) and early patient data confirm resistance can emerge — via novel KRAS mutations in cis/trans, other RAS paralogs (NRAS,

HRAS), upstream receptors (EGFR), or downstream pathway mutations. Daraxonrasib has a dual mechanism (steric hindrance + GTP hydrolysis stimulation) that may delay resistance, but combinations are likely needed.[10][15]

4. **NSCLC competitive intensity:** In NSCLC, daraxonrasib faces a crowded G12C field (olomorasib, divarasib, calderasib, adagrasib, sotorasib) and will need to differentiate on pan-RAS coverage (non-G12C patients) and efficacy. The NSCLC market is larger than PDAC but more competitive.[8][18]
5. **Commercial execution at \$39,800/month:** Payer access and reimbursement restrictions are a material risk; payer pressure on multi-billion oncology spending may favor biomarker-stratified prescribing and combination cost optimization.[6][8]
6. **Peak-sales dependency:** RVMD's ~\$45B market cap and \$11.5B peak-sales estimate rest heavily on daraxonrasib. If the 1L PDAC readouts or NSCLC data disappoint, the valuation is vulnerable. The company has no commercial revenue yet.[6][7]
7. **Chinese KRAS ecosystem maturation:** Four NMPA-approved G12C inhibitors plus emerging G12D programs mean Chinese players will increasingly compete in emerging markets and through licensing deals — potentially compressing pricing and limiting ex-US expansion opportunities.[8]
8. **Long-term modality disruption:** KRAS vaccines (Moderna, Elicio, BioNTech) and TCR-T cell therapies could transform KRAS management by 2030, compressing the long-term window for small-molecule monotherapy.[8][9]

9. Strategic Outlook (Next 24-36 Months)

The field's milestones that will determine daraxonrasib's competitive trajectory:[8][9]

- **Daraxonrasib Phase 3 PDAC readouts:** RASolute 302 (2L, complete — approved), RASolute 309 (1L G12D combo with zoldonrasib), and 1L all-comers monotherapy trial. The 1L data will determine whether daraxonrasib moves from a 2L option to the backbone of first-line PDAC treatment.
- **Setidegrasib Phase 3 KRAS G12D PDAC readout (Astellas):** The first PROTAC degrader in Phase 3; if positive, creates a differentiated competitor in the largest PDAC allele segment. Readout expected 2026-2027.
- **NSCLC Phase 3 readouts:** Daraxonrasib's 2L/3L NSCLC trial will define its position vs the G12C incumbents. Simultaneously, olomorasib, divarasib, and calderasib Phase 3 1L NSCLC data (expected 2026-2027) will reshape the G12C standard of care.
- **mRNA-4157 melanoma adjuvant Phase 3 (Moderna/Merck):** A bellwether for the vaccine modality — if positive, accelerates the KRAS vaccine timeline.
- **AbbVie-Jacobio Glecirasib ex-Asia commercial scaling:** Validates Chinese KRAS asset global commercial potential.
- **Chinese KRAS G12D ecosystem maturation:** Hengrui HRS-4642 and others advancing — could bring competitive G12D options to market at lower price points.

Most likely trajectory: Daraxonrasib establishes a dominant position in 2L+ metastatic PDAC (no targeted competition for 12-24 months). The 1L PDAC opportunity is the real commercial prize — and it's a head-to-head battle with Astellas' setidegrasib (G12D PROTAC) and Revolution's own zoldonrasib combo. NSCLC is a larger but more contested market where pan-RAS coverage is the differentiator. Vaccines and TCR-T are a 2030 threat, not a near-term risk.

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Report generated September 12, 2026. All claims are sourced; where a claim rests on a single source, it is noted. Competitive landscape is dynamic — the 1L PDAC and NSCLC Phase 3 readouts over the next 12-24 months will materially reshape the field.

Sources and Method

This analysis was prepared exclusively from publicly available information.

Source class	Examples used
Regulatory	FDA approvals and label documents, NMPA approvals, EMA
Trial registries	ClinicalTrials.gov (NCT identifiers cited inline)
Peer-reviewed literature	NEJM, ASCO/JCO, Frontiers in Oncology, PMC/NIH
Company disclosure	Revolution Medicines IR and pipeline pages, Astellas newsroom
Industry analysis	Eureka/Patsnap landscape analyses, trade press

No non-public, confidential, or employer-derived information was used or solicited in the preparation of this analysis. No primary research, expert interviews, or insider sources were used. Where a claim rests on a single source, it is noted in the text.

Method: continuous monitoring across the five source classes above, triage for materiality, trial-level analysis, competitive positioning, and decision-ready synthesis. Every material claim carries a numbered citation — 25 sources in total.

This deliverable is competitive intelligence prepared from public sources. It is not legal, regulatory, medical, investment, or tax advice. The competitive landscape is dynamic; the readouts identified in Section 9 will materially reshape this field.

Prepared as a portfolio sample by VigilLine. No client engagement underlies it.